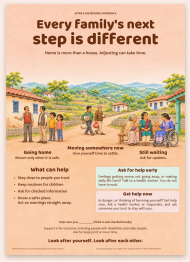
Talking about: Going home, moving or waiting



For people trained in psychological first aid, with a supervisor and known referral routes.

Before the talk, read the facilitator pages from page 3.

> **Immediate danger?** Pause the session, stay with the person and get urgent help now. Do not leave them alone. Stay until a health worker or responder takes over; feeling calmer is not the end point. Full procedure and contacts: page 2.



Poster A3



Leaflet, tri-fold

Your materials

Poster · a shared focus
For a group: open the discussion, invite observations and return to the main message. Make sure everyone can see; do not read every line aloud.

Leaflet · detail to take home
For a question or a closer look, and to keep or share. Open only the relevant section, for example the path closest to a family, and offer to read it together.

Use one or both, in any order. Focus on what people need, not on finishing both materials.

Before people arrive

- ☐ Check the latest local information: who to ask about house safety checks, assistance and warnings, and when they can be reached. What the ward office does differs by area; confirm it locally.
- ☐ Fill in and check the help contacts on page 2. Hang the poster where everyone can see it; bring enough leaflets. Plan time after the session for private talks.
- ☐ Plan language and access: simple words, large print, more time, or a sign language interpreter if needed and available. Offer alternatives if the picture or text is not accessible.

The session

about 15 minutes · do not rush someone or delay urgent help to finish the plan

Aim: each family thinks about its own next step, knows where to ask for checked information, and knows where to get help.

1 Welcome and explain the purpose

2 min · either material

DO Introduce yourself and the topic. Joining is voluntary. Nobody has to say where they are going or why.

SAY *"Every family's situation is different. Today we will talk about what can help, wherever you are."*

2 Look at the picture together

3 min · poster if useful

DO Point to the three families: going home, moving somewhere new, still waiting. Let people choose which is closest to them, silently if they prefer. With the leaflet only, use its three paths.

ASK *"Which family in the picture is closest to yours now?"*

3 Feelings

3 min · either material

DO Explain that relief, sadness, worry or anger can come together, and that home is more than a building.

SAY *"Missing neighbours, work and memories is part of this. These losses matter."*

ASK *"What do people here miss most?"*

4 What can help

4 min · move between materials as needed

DO Go through *"What can help"* on the poster. Share only local information you have checked.

ASK *"Who will you ask for information this week? What would you do if a warning comes?"*

5 Check understanding and close

3 min · offer the leaflet to keep

DO Give out the leaflet and show *"You do not have to manage alone"* and the help messages. Tell people where to find you after the session.

ASK *"If someone needs help today, where could they go or who could they contact?"*

Safety, referral and worker boundaries

Get urgent help now if someone:

SIGNS	WHAT TO DO
• Talks about suicide, or about harming themselves or someone else	1. Stay with the person. Do not leave them alone.
• Cannot keep themselves safe, or cannot care for a child in their care	2. Get urgent help now: a health worker, responder or the emergency service in this area.
• Seems very confused, or not aware of where they are	3. It is okay to ask directly about suicide. Asking does not put the idea in their mind.
	4. Stay until a health worker or responder takes over. Feeling calmer is not the end point.

If someone becomes upset

- Stay calm and stay close.
- Offer a quieter place away from the group, if they wish.
- Do not force them to talk. Ask what they need now.
- Stay as long as they want, or until someone they trust is with them.

PFA: Look, Listen and Link

Look: safety, urgent needs, people who are very distressed
Listen: ask about needs and concerns; help them feel calm
Link: basic needs, services, information, loved ones

Talk privately and link to further help if

- Feelings get worse, do not improve, or make daily life very hard
- They are not able to look after themselves or their children
- A body problem worries them: pain, fever, injury, or not eating or sleeping for days
- Practical needs are not met: food, water, shelter, medicines or care. Help them contact the relevant service; do not assume this is a mental health problem

With the person's agreement: explain the options, go with them if you can, and share only what is needed.

Do

- ✓ Use simple words and respect silence
- ✓ Give only checked local information
- ✓ Respect each family's own decision
- ✓ Follow up privately with anyone who asks

Do not

- ✗ Promise housing, money or dates
- ✗ Tell families whether they should go home or not
- ✗ Push people to talk about what they lost
- ✗ Pass on information you have not checked

Help contacts for this area fill in and check before the talk

Health worker or facility _____

Urgent help _____

House safety checks _____

Information on assistance _____

Warnings and a safer place _____



Sources: WHO, Psychological First Aid: Guide for Field Workers (2011); IASC, Guidelines on MHPSS in Emergency Settings (2007), Action Sheets 8.1–8.2; IFRC Psychosocial Centre, Mental health and psychosocial impacts of flooding. This material draws on these publications; it was not created by WHO, IASC or IFRC, which are not responsible for its content.

What this material is about

The poster and leaflet use everyday words, so that people can talk about going home, moving or waiting without clinical labels. This page gives the meaning behind those words and the limits of your role; page 4 holds the clinical terms, for reference only.

Word key

ON THE POSTER OR LEAFLET	WHAT WE MEAN	HOW TO HANDLE IT IN THE TALK
***"Every family's next step is different"*: going home, moving somewhere new, still waiting**	Return, relocation and continued displacement, all valid outcomes	Do not rank the paths or advise a family which to choose
"Home is more than a house"	Loss of home, place, community and livelihood, which can be grieved	Acknowledge these losses; do not minimise them because the house still stands
"Ask for checked information"*; *"Ask for updates"	Accurate, timely information reduces distress; uncertainty itself is a stressor	Share only verified information; never promise housing, money or dates

Feelings after a move, and when to look further

Common feelings

Relief, sadness, worry or anger, sometimes all at once. Missing neighbours, work, familiar places and memories. Not knowing what comes next is hard in itself.

Early help, at any time

If feelings get worse, do not go away, or make daily life hard, link the person to a health worker. They do not have to wait.

Get help now

If there is danger, follow page 2 at once: stay with the person and get urgent help.

Keep these apart

- **Practical needs** (food, shelter, documents or money) are linked to services, not read as a psychological reaction.
- **Distress from loss and uncertainty** is a common response, not an illness.
- **A diagnosis** describes a lasting pattern that impairs daily life, assessed by a trained health worker.

Your limits

- You do not advise a family whether to go home, move or wait.
- You do not promise housing, money or dates.
- You share only information you have checked. Who does house checks and who handles assistance is confirmed locally.

Your role

This guide supports a session; it does not replace psychological first aid training. Use your training, supervisor and known referral routes.

Look after yourself too: take breaks, talk with your supervisor or a colleague, and ask for support when you need it.



Talking privately about a next step

- Introduce yourself and ask permission. Explain what you can and cannot offer.
- Help with one need they choose: checked information, a practical service, or a person they trust.
- Agree any follow-up only if it is possible and wanted; do not promise a visit or call you cannot make.
- Listen to what the person misses and what worries them most. These losses matter.
- If medicines are lost or running out, link them to a health worker to continue them safely.

Sources: WHO, Psychological First Aid: Guide for Field Workers (2011); IASC, Guidelines on MHPSS in Emergency Settings (2007), Action Sheets 8.1–8.2; IFRC Psychosocial Centre, Mental health and psychosocial impacts of flooding.

Clinical terms you may hear

For reference only. Do not use these terms with participants, and do not diagnose. They help you understand what a health worker may say, and why practical help comes first.

TERM OR SITUATION	WHAT THE FACILITATOR NEEDS TO KNOW	SOURCE
Stress of displacement and uncertainty	Loss of home, place and livelihood, and not knowing what comes next, are major ongoing stressors. Accurate information and practical help reduce distress.	IFRC Psychosocial Centre, Mental health and psychosocial impacts of flooding; IASC (2007)
Adjustment disorder	A maladaptive reaction to an identifiable stressor that usually emerges within a month and typically resolves within six months, unless the stressor persists. Do not label people; where stressors continue, practical support comes first. Assessed only by a trained health worker.	WHO, ICD-11, 6B43
Practical needs are not mental-health problems	Lack of food, shelter, documents or money is linked to services, not interpreted as a psychological reaction.	IASC, Guidelines on MHPSS in Emergency Settings (2007)
***"Trauma"*, when a participant uses the word**	Do not correct the word and do not start a discussion about trauma. Ask what the person means: *"When you say trauma, what do you mean? What do you notice in yourself?"* People often use the word for a specific fear or change, for example *"I am afraid to go back home because it might happen again"*. Such fear is a common reaction after a frightening or potentially traumatic experience. Respond to what the person describes, and use the early-help route if it continues, gets worse or makes daily life hard.	Facilitation practice for this series; listening as in WHO, Psychological First Aid (2011)

Practice cards

Examples of what people may say and what you can do, step by step. They take place in a private conversation, not in front of the group. The words are an illustration: use your own, follow the person's lead, and use page 2 at once if there is any urgent sign.

Words for you to say or ask · What the person may say · Guidance for you (plain text)

WHEN SOMEONE SAYS "TRAUMA"

"I have trauma. I cannot go back to my house."

Poster: going home, return only when it is safe.

1 Ask what they mean

ASK *"When you say trauma, what do you mean? What do you notice in yourself?"*

THEY SAY *"When I get near the house my heart beats fast. I keep thinking it will happen again."*

2 Name it as common

SAY *"Feeling afraid near the place where something frightening happened is a common reaction."*

3 Keep fear and safety apart

DO If the house has not had a safety check, the advice is still not to go in. Help them find out who does house checks in this area.

4 Know when to look further

DO If the fear gets worse, does not go away, or makes daily life hard, link the person to a health worker. They do not have to wait.

WHEN A FAMILY ASKS YOU TO DECIDE

"Should we go home or stay here? You tell us."

Poster: every family's next step is different.

1 Do not decide for them

SAY *"This is your family's decision. I can help you find checked information, so that you can decide."*

2 Find out what they need to know

ASK *"What do you need to know to decide? Who could you ask?"*

3 Link to checked information

DO Give only checked contacts for house checks, assistance or warnings. Never promise housing, money or dates.

WHEN WAITING FEELS ENDLESS

"Nobody tells us anything. We have been waiting for weeks."

Leaflet: still waiting.

1 Acknowledge

SAY *"Not knowing what comes next is hard."*

2 Prepare questions together

DO Help them ask the person in charge of the shelter or the ward office: *"What is known? What is not decided yet? When can I ask again?"*

3 Check practical needs

ASK *"Is anything missing today, such as food, water or medicines?"*

DO Help them contact the relevant service; do not treat a practical need as a mental-health problem.